

Department of Neuro Surgery
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 48.0 Yr/M
Admission No.: anonymous	Department Unit: Neuro Surgery (Ns Unit 1)	Consultant/Faculty: anonymous
Ward: 3 B 2 Neuro Surgery Ward	Room/Bed: 3 B 2 Neuro Surgery/ 3 B 2 N S 12	Patient Category: General
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation:
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 14-Mar-2023 18:40	D.O.D. : 04-Apr-2023	D.O. Print Report: 08-Feb-2026 15:39

FINAL DIAGNOSIS:

Acquired defect of skull;

Remarks: PREVIOUSLY OPERATED CASE OF LEFT DECOMPRESSIVE CRANIECTOMY WITH SKULL DEECT

OPERATION PERFORMED:

AUTOLOGOUS BONE FLAP CRANIOPLASTY DONE UNDER On 03-Apr-2023

SURGEONS: Dr Binoy Kumar Singh

OPERATIVE FINDINGS: ABDOMINAL INCISION REEXPLORED AND BONE FLAP RETRIEVED. CRANIAL END REEXPLORED, SUBCUTANEOUS FLAP DISSECTED FROM DURA/ PERICRANIUM GRAFT. BONE FLAP FIXED WITH 4 MINI PLATES AND 8 SCREWS (TITANIUM, MRI COMPATIBLE JAYON). INCISION CLOSED IN LAYERS.

HISTORY OF PRESENT ILLNESS: A/H/O INJURY OVER HEAD 6 MONTHS BACK MANAGED BY DECOMPRESSION CRANIECTOMY WITH ABDOMINAL FLAP PLACEMENT IN OUTSIDE HOSPITAL (PREVIOUS DOCUMENTS NOT AVAILABLE)

PAST HISTORY : K/C HYPERTENSION SINCE 8 YEARS ON MEDICATIONS

CNS EXAMINATION: - CONSCIOUS ORIENTED TO TIME, PLACE AND PERSON. OBEYS VERBAL COMMANDS, GCS- E4V5M6

PUPIL- B/L SYMMETRICAL AND REACTING TO LIGHT.

CRANIAL NERVES EXAMINATION: - EXAMINATION IS WITHIN NORMAL LIMITS

MOTOR - BULK - NO WASTING OR ANY FASCICULATION, TONE - NORMAL IN BOTH UPPER AND LOWER LIMB, POWER - UPPER LIMB - SHOULDER - ABD/ADD - RT- 5/5, LT- 5/5, ELBOW - FLEXION/ EXTENSION - RT - 5/5, LT- 5/5, WRIST- RT -5/5, LT-5/5 ,LOWER LIMB - HIP - FLEXION/ EXTENSION - RT- 5/5, LT- 5/5, KNEE- FLEXION/ EXTENSION - RT - 5/5, LT- 5/5, ANKLE - DF/PF - RT- 5/5, LT- 5/5, EHL-RT-5/5, LT-5/5, DTR-RT- 2 , LT-2 .

PLANTAR- B/L FLEXOR

SPEECH - CLEAR AND FLUENT

MEMORY - INTACT

INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
17-Mar-2023 01:29	GLYCOSYLATED HAEMOGLOBIN (HbA1c)	Result	Refer Reports*	-
		Hba1c	5.8	< 5.7 %

*Refer Lab Reports for details.

OTHER INVESTIGATIONS: PRE OP NCCT HEAD S/O LEFT FTP SKULL DEFECT



C-DAC: Centre for Development of Advanced Computing
C-56/1, Sector-62, Noida - 201307, Uttar Pradesh,
91-0120-3063311-13.



HOSPITAL COURSE: PATIENT GOT ADMITTED WITH ABOVE MENTIONED COMPLAINTS ALL RELEVANT INVESTIGATIONS WERE DONE AND DAIGNOSED AS A CASE OF PREVIOUSLY OPERATED CASE OF LEFT DECOMPRESSIVE CRANIECTOMY WITH SKULL DEEFCT. THE PATIENT UNDERWENT AUTOLOGOUS BONE FLAP CRANIOPLASTY DONE UNDER GA ON 3/4/23. POST OPERATIVE PERIOD WAS UNEVENTFUL. NO NEW NEUROLOGICAL DEFICITS NOTED. OVERTIME PATIENT GENERAL CONDITION IMPROVED. PATIENT IS DISCHARGED WITH STABLE VITALS AND SENSORIUM.

CONDITION AT DISCHARGE: PATIENT CONSCIOUS, ORIENTED WITH ALL STABLE VITALS, GCS- 15/15. POWER – B/L UPPER LIMB AND LOWER LIMB – 5/5

WOUND – HEALTHY, SUTURE IN SITU

ADVICE ON DISCHARGE:

- DAILY BATH, MAINTAIN HYGIENE
- TAB PCM 650MG SOS
- TAB PANTOP 40 MG OD FOR 5 DAYS
- TAB AMLODIPINE 5 MG OD TO CONTINUE

DISCHARGE BY : anonymous

DISCHARGE REMARKS : IN CASE OF ANY EMERGENCY LIKE HEADACHE, WOUND INFECTION/ DISCHARGE, WEAKNESS REVIEW IN EMERGENCY TRAUMA OR IN NEUROSURGERY OPD.

FOLLOW UP ON : 11-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE : REVIEW AFTER ONE WEEK IN NEUROSURGERY OPD AND SUTURE TO BE REMOVED ON 11/4/23.

Summary Prepared By

Summary Approved By

anonymous

anonymous